



FruzaqlaTM
(fruquintinib) capsules

5 mg • 1 mg

Day 1 Look and Feel Style Guide

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Brand personality

The brand personality represents the emotional qualities and underlying spirit that the brand embodies. FRUZAQLA is fearless, inspiring, influential, and approachable.

Fearless

FRUZAQLA is FEARLESS, unafraid to push boundaries and confident in what we have to offer

Inspiring

FRUZAQLA is INSPIRING and exciting as it reinvigorates interest and enthusiasm

Influential

FRUZAQLA is INFLUENTIAL and forward looking as a practice-changing difference maker for the future

Approachable

FRUZAQLA is APPROACHABLE, friendly, open, and welcoming

Hallmark

These graphic elements provide the foundation for all promotional materials, both in print and digital. Circular shapes are combined with straight lines—in both solid and gradient expressions—to convey motion, power, and inspiration.



Rubik

A B C D E F G H I J K L M N

O P Q R S T U V W X Y Z

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q r s t u v w x y z

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Rubik Light

Rubik Light Italic

Rubik Regular

Rubik Regular Italic

Rubik Medium

Rubik Medium Italic

Rubik Bold

Rubik Bold Italic

Rubik Black

Rubik Black Italic



Special font use

Roboto Condensed is the font used when the page is copy heavy and it is necessary to maximize space.

6

Fruzaqla[®]
(fruquintinib) capsules
5 mg • 1 mg

BRIEF SUMMARY OF PRESCRIBING INFORMATION

These highlights do not include all of the information needed to use FRUZAQla safely and effectively. See full prescribing information for FRUZAQla.

FRUZAQla (Fruquintinib) capsules, for oral use. **U.S. Approval: 2022**

INDICATIONS AND USAGE

FRUZAQla is indicated for the treatment of adult patients with metastatic colorectal cancer (mCRC) who have been previously treated with fluoropyrimidine, oxaliplatin, and irinotecan-based chemotherapy, an anti-VEGF therapy, and, if not adequate, and metastatically appropriate, an anti-EGFR therapy.

WARNINGS AND PRECAUTIONS

5.1 Hypertension

FRUZAQla can cause hypertension. Hypertension occurred in 45% of 911 (44%) patients with mCRC treated with FRUZAQla, including Grade 3 or 4 events in 11%, and hypertension crisis in three patients (0.3%). The median time to first onset of hypertension was 14 days from first dose of FRUZAQla.

To maintain FRUZAQla's antineoplastic blood pressure is adequately controlled. Monitor blood pressure weekly the first month, at least monthly thereafter and as clinically indicated. Initiate or adjust antihypertensive therapy as appropriate. Withhold, reduce dose, or permanently discontinue FRUZAQla based on the severity of hypertension.

5.2 Hemorrhagic Events

FRUZAQla can cause serious hemorrhagic events, which may be fatal. In 911 patients with mCRC treated with FRUZAQla, 15% of patients experienced a gastrointestinal hemorrhage, including 13 patients (1%) with a Grade 3 or 4 event and 2 patients with fatal hemorrhage. Permanently discontinue FRUZAQla in patients with severe or life-threatening hemorrhage. Monitor the International Normalized Ratio (INR) levels in patients receiving antiplatelets.

5.3 Infection

FRUZAQla can cause an increased risk of infections, including fatal infections. In 781 patients treated with FRUZAQla across three randomized, placebo-controlled trials, the overall incidence of infections was higher (37% vs. 27%) including for fatal infections (1% vs. 0.5%) as compared to the placebo arm (n=391). In 911 patients with mCRC treated with FRUZAQla, the most common infections were urinary tract infections (8.1%), upper respiratory tract infections (7.2%) and pneumonia (2.5%). Fatal infections included pneumonia (0.8%), sepsis (0.5%), bacterial infection (0.1%), lower respiratory tract infection (0.1%), and septic shock (0.1%). Withhold FRUZAQla for Grade 3 or 4 infections, or worsening infection of any grade. Resume FRUZAQla at the same dose when the infection has resolved.

5.4 Gastrointestinal Perforation

FRUZAQla can cause gastrointestinal perforation. In 911 patients with mCRC treated with FRUZAQla, 1.2 patients (0.1%) experienced a Grade 3 or 4 gastrointestinal perforation, including one fatal event.

Permanently discontinue FRUZAQla in a patient who develops gastrointestinal perforation.

5.5 Neutropenia

FRUZAQla can cause hair loss. In 911 patients with mCRC treated with FRUZAQla, 48% experienced increased ALT or AST, including Grade 3 or 4 events in 5%, and fatal events in 0.2%. Median time to first onset of elevated liver enzymes was 29 days from first dose of FRUZAQla.

Monitor liver function tests (ALT, AST, and bilirubin) before initiation and periodically throughout treatment with FRUZAQla. Temporarily hold and then reduce or permanently discontinue FRUZAQla, depending on the severity and persistence of hepatotoxicity as manifested by elevated liver function tests.

5.6 Proteinuria

FRUZAQla can cause proteinuria. In 911 patients with mCRC treated with FRUZAQla, 38% experienced proteinuria and 13% of patients experienced Grade 3 or 4 events. Median time to first onset of proteinuria was 22 days from first dose of FRUZAQla.

Monitor for proteinuria before initiation and periodically throughout treatment with FRUZAQla. For proteinuria Grade 3 or 4 events, withhold FRUZAQla until improvement in Grade 3 proteinuria, resume FRUZAQla at a reduced dose. Discontinue FRUZAQla in patients who develop proteinuria Grade 4.

5.7 Palm-Plant Erythroedema (PPE)

FRUZAQla can cause PPE. In 911 patients with mCRC treated with FRUZAQla, PPE occurred in 35%, including 8% with Grade 3 events. Median time to first onset of PPE was 19 days from first dose of FRUZAQla.

5.8 Palm-Plant Erythroedema (PPE)

FRUZAQla can cause PPE. In 911 patients with mCRC treated with FRUZAQla, PPE occurred in 35%, including 8% with Grade 3 events. Median time to first onset of PPE was 19 days from first dose of FRUZAQla.

Based on severity, withhold FRUZAQla and then resume at the same or reduced dose.

5.9 Palm-Plant Erythroedema (PPE)

FRUZAQla can cause PPE. In 911 patients with mCRC treated with FRUZAQla, PPE occurred in 35%, including 8% with Grade 3 events. Median time to first onset of PPE was 19 days from first dose of FRUZAQla.

Based on severity, withhold FRUZAQla and then resume at the same or reduced dose.

wound/healing. The safety of resumption of FRUZAQla after resolution of wound healing complications has not been established.

5.10 Arterial Thromboembolic Events

FRUZAQla may increase the risk of arterial thromboembolic events. In 911 patients with mCRC treated with FRUZAQla, 7 patients (0.8%) experienced an arterial thromboembolic event; additionally, FRUZAQla studies excluded patients with clinically significant cardiovascular disease, uncontrolled hypertension, or with thromboembolic events within the prior 6 months. Initiation of FRUZAQla in patients with a recent history of thromboembolic events should be carefully considered. In patients who develop arterial thromboembolic events, discontinue FRUZAQla.

5.11 Allergic Reactions to FRUZAQla Yellow No. 5 (Tartrazine) and No. 6 (Sunset Yellow FCF)

FRUZAQla is a yellow capsule containing FRUZAQla Yellow No. 5 (Tartrazine) and No. 6 (Sunset Yellow FCF). FRUZAQla is a yellow capsule containing FRUZAQla Yellow No. 5 (Tartrazine) and No. 6 (Sunset Yellow FCF). FRUZAQla is a yellow capsule containing FRUZAQla Yellow No. 5 (Tartrazine) and No. 6 (Sunset Yellow FCF).

6 ADVERSE REACTIONS

The following clinically significant adverse reactions are described elsewhere in the labeling:

• Hypertension

• Hemorrhagic Events

• Infection

• Gastrointestinal Perforation

• Neutropenia

• Proteinuria

• Palm-Plant Erythroedema (PPE)

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Roboto Condensed

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Roboto Condensed Light

Roboto Condensed Light Italic

Roboto Condensed Regular

Roboto Condensed Regular Italic

Roboto Condensed Bold

Roboto Condensed Bold Italic

Fruzaqla[™]
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Hierarchy

The hierarchy and prominence of copy will dictate its specific style.

FRESCO-2 Study

In FRESCO-2, the majority of ARs were manageable and predictable^{1,4,16}

ARs occurring in $\geq 10\%$ of patients¹

AR O	FRUZAQLA + BSC (n=456)		Placebo + BSC (n=230)	
	All grades (%)	Grades 3/4 (%)	All grades (%)	Grades 3/4 (%)
Fatigue*	63	12	39	6.8
Hypertension*	38	14	9	0.9
Stomatitis*	31	2.2	7.8	0.4
Abdominal pain*	25	3.5	20	3
Diarrhea*	24	3.7	11	0
Hypothyroidism	21	0.4	0.4	0
Palmar-plantar erythrodysesthesia	19	6	2.6	0
Proteinuria*	18	1.8	5	0.9
Dysphonia*	18	0	5	0
Musculoskeletal pain*	16	1.1	7	0
Arthralgia	11	0.9	4.3	0

*Represents a composite of multiple related terms.

¹ Predictable refers to ARs consistent with inhibition of VEGF and VEGFRTM

⁴ Serious ARs occurred in 38% of patients treated with FRUZAQLA + BSC. Serious ARs in $\geq 2\%$ of patients treated with FRUZAQLA + BSC included hemorrhage (2.2%) and gastrointestinal perforation (2.0%)¹

¹⁶ Fatal ARs occurred in 14 (3.1%) patients treated with FRUZAQLA + BSC. Fatal ARs occurring in ≥ 2 patients treated with FRUZAQLA + BSC include pneumonia (n=3), sepsis/septic shock (n=2), and hepatic failure/encephalopathy (n=2)

AR=adverse reaction.

*Despite predictability, individual patient experiences may vary.

Please see additional Important Safety Information throughout, full Important Safety Information on pages 23-24, and enclosed FRUZAQLA full Prescribing Information in pocket.

Headlines use Rubik Bold in dark blue and are sentence case

Chart heads use Rubik Bold in purple and are sentence case

Column head uses Rubik Bold in purple

Column head uses Rubik Bold in 80% black

Bold body copy uses Rubik Bold in dark blue

Body copy for placebo uses Rubik Regular in 80% black

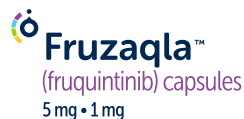
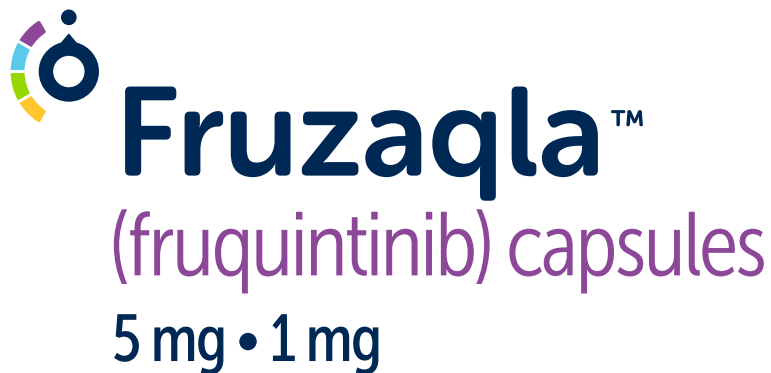
Bold body copy for FRUZAQLA column uses Rubik Bold in purple

Bullet point uses Rubik Black in light blue

Bullet point body copy uses Rubik Regular in dark blue

Logo and approved uses

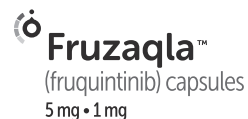
The horizontal logo is the sole logo mark. It should never be manipulated or tampered with beyond its original design. Typeface and colors should be used accurately and consistently and the logos are to be used as stipulated in this guide, without alterations in design, color, and aspect ratio.



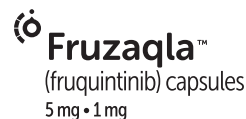
COLOR LOGO



5 COLOR PMS LOGO



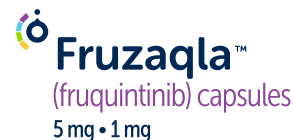
GRAYSCALE LOGO



BLACK & WHITE LOGO



KNOCKOUT LOGO



Logo clearance

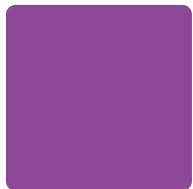
When accounting for clearance space, use the height of the cap “F” to establish the surrounding space. Maintain this space between the logo and other content for clarity and legibility.



Color palette and hierarchy

Exact brand colors should be consistent and appear across all promotional materials.

Color palette



Pantone 258 C
 CMYK: 51, 79, 0, 0
 RGB: 140, 71, 153
 HEX: 8C4799



Pantone 305 C
 CMYK: 54, 0, 6, 0
 RGB: 89, 203, 232
 HEX: 59CBE8



Pantone 295 C
 CMYK: 100, 69, 8, 54
 RGB: 0, 40, 85
 HEX: 002855



Pantone 375 C
 CMYK: 46, 0, 90, 0
 RGB: 151, 215, 0
 HEX: 97D700



Pantone 123 C
 CMYK: 0, 19, 89, 0
 RGB: 255, 199, 44
 HEX: FFC72C

Color hierarchy

This is how the brand colors should be used in all promotional materials.
 Colors outside of this palette should not be used.



**Primary
Color**



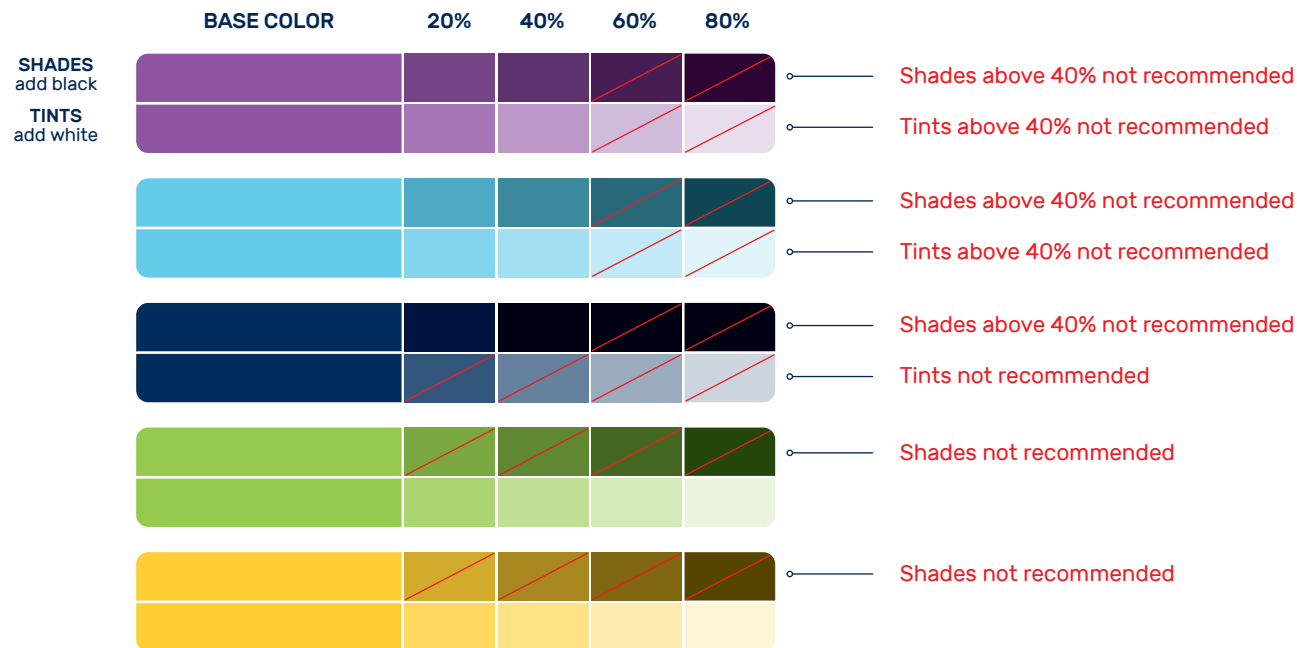
**Secondary
Color**



**Tertiary
Colors**

Color tints and shades

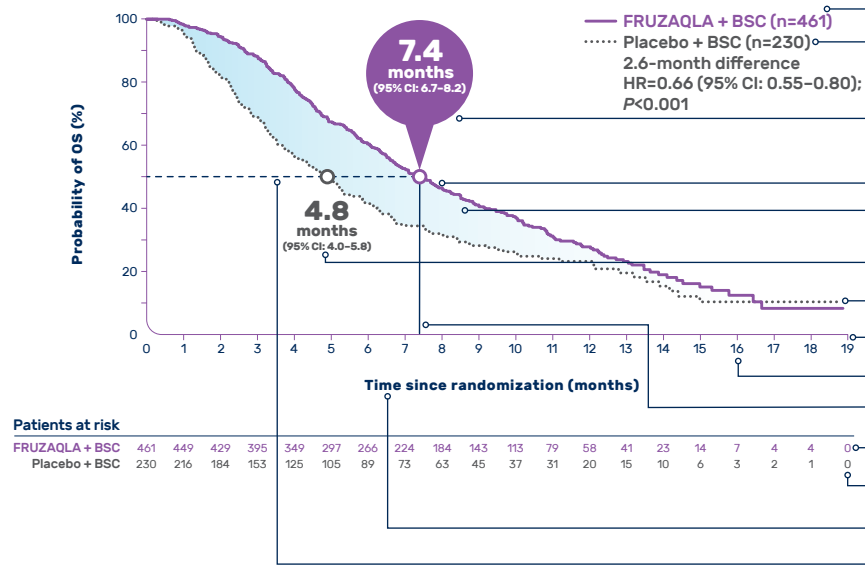
These are guidelines for color tints and shades. Avoid using shades and tints above 40% and tints below 20%.



Charts

These charts are designed specifically for FRUZAQLA and should not be modified.

OS in patients with previously treated mCRC



1-point line weight in purple

Rubik Bold in purple

Rubik Medium in purple

Rubik Medium in 80% black

Use this graphic element when highlighting key data for FRUZAQLA

2-point line weight in purple

Gradient fill is light blue to white

Placebo data point uses Rubik Bold in 80% black

2-point dotted line weight in 80% black

.5-point line weight in purple

Rubik Regular in dark blue

1-point line weight in dark blue

Rubik Regular in purple

Rubik Regular in 80% black

Rubik Bold in dark blue

1-point dash line weight in dark blue

Tables

These tables are designed specifically for FRUZAQLA and should not be modified.

ARs occurring in ≥10% of patients¹

AR	FRUZAQLA + BSC (n=456)		Placebo + BSC (n=230)	
	All grades (%)	Grades 3/4 (%)	All grades (%)	Grades 3/4 (%)
Fatigue ^a	53	12	39	4.8
Hypertension ^a	38	14	9	0.9
Stomatitis ^a	31	2.2	7.8	0.4
Abdominal pain ^a	25	3.5	20	3
Diarrhea ^a	24	3.7	11	0
Hypothyroidism	21	0.4	0.4	0
Palmar-plantar erythrodysesthesia	19	6	2.6	0
Proteinuria ^a	18	1.8	5	0.9
Dysphonia ^a	18	0	5	0
Musculoskeletal pain ^a	16	1.1	7	0
Arthralgia	11	0.9	4.3	0

1-point line weight in purple

Chart head uses Rubik Bold in purple

Column head uses Rubik Bold in purple

Column head uses Rubik Bold in 80% black

Bold body copy uses Rubik Medium in dark blue

Rubik Bold in dark blue

Rubik Bold in purple

Rubik Regular in 80% black

.5-point line weight in 20% tint

Light blue row uses 90% tint

Violators

These brand elements show the use of the Now Approved and secondary page violators and should not be modified.



Light blue

Text should be yellow

Dark blue/purple gradation

Green

Dark blue/purple gradation

NOW APPROVED: An innovation in mCRC

Text should be white, yellow, light blue, or green

Iconography

These are guidelines for creating new icons.



Icons should be light and predominantly white, using the brand purple as the contrast

2-point line weight in purple

Avoid sharp edges to keep art rounded and soft, especially for arrows and boxes

2-point line weight in blue, green, and yellow gradation stroke frames the icons

For branding and visual impact, Rubik Bold and Rubik Medium is used

Icons have a subtle light blue drop shadow to add depth without overpowering the design

Iconography, cont'd

The following icons are shared across all patient and HCP materials and should be used consistently for visual emphasis and support of messages.



Clinical
Studies



Safety and
Efficacy



More
Time



Once
a Day



34% Reduction
in Risk



21 Days



First in a
Decade



Support



With or
Without Food



Discuss With
Your Doctor



Clinical
Data



Discover
More



Dosing



Press
Release



Strength



3.7 Months

Callouts and CTAs

These are guidelines for calling out data, key messaging, and call to action.

**XX% Erspiclati
antium**

Sed ut perspiciatis unde omnis
HR=0.XX (XX% CI: 0.XX-0.XX)

Key data point uses Rubik Bold in purple

Supporting subhead uses Rubik Bold in purple

Body copy uses Rubik Regular in dark blue

Inner arc uses a gradient line made up of purple, light blue, green, and yellow

Outer arc is shown in purple with rounded end caps

Overall example of key messaging layout

Finibus Bonorum et Malorum

Sed ut perspiciatis unde omnis nat Excepteur sint occaecat cupidatat non proident,
sunt in culpa qui officia deserunt mollit anim id est laborum tus error sit ltm

visit **FRUZAQLA**hcp.com



Icon uses gradient lines made up of yellow, light blue, and green

Supporting subhead uses Rubik Medium in purple



QR code should always be in purple

 **Fruzaqla™**
(fruquintinib) capsules
5 mg • 1 mg

These are guidelines for the visual presentation of key information and data.

[illegible]

Copy should always appear on a white background



62% Disease control rate
with FRUZAOLA + BSC



Key data uses purple

20% black

Key data uses 80% black

20% black



NOTE: Pieces might be different in size. If the pieces are smaller please maintain the font size ratios on the ISI.

Now Approved
An innovation
in mCRC

Fruzaqla™
(fruquintinib) capsules
5 mg • 1 mg

**The first and only novel
targeted therapy approved for
mCRC in more than a decade¹⁻⁵**

Study design subhead uses a Rubik Bold in dark blue and is sentence case (7 pt)

INDICATION HEAD USES RUBIK MEDIUM IN PURPLE AND IN ALL CAPS (11 pt)

IMPORTANT SAFETY INFORMATION HEADS USE RUBIK MEDIUM IN PURPLE (11 pt)

WARNINGS AND PRECAUTIONS USES RUBIK BOLD IN DARK BLUE AND IN ALL CAPS (10 pt)

ISI Sub-section Headers Use Rubik Bold in Dark Blue and Title Case (10 pt)

Bullet points use Rubik Black in light blue (10 pt)

Body copy uses Rubik Regular in dark blue and is sentence case (8.7 pt)

For patients who have received oxaliplatin- and irinotecan-based regimens, regardless of mutation status

Improved survival¹
7.4 months (95% CI: 6.7-8.2) with FRUZAQLA + BSC vs 4.8 months (95% CI: 4.0-5.6) with placebo + BSC, a 2.6-month difference
HR=0.44 (95% CI: 0.35-0.55), P<0.001

68% IMPROVED TIME TO PROGRESS AND OVERALL SURVIVAL

56% MAINTAINED STABLE DISEASE

Manageable safety profile^{1,4}
20% discontinuation rate due to AEs with FRUZAQLA + BSC vs 21% with placebo + BSC

More than doubled mPPS^{1,4}
3.7 months (95% CI: 3.5-3.8) with FRUZAQLA + BSC vs 1.8 months (95% CI: 1.6-1.9) with placebo + BSC, a 19-month difference
HR=0.32 (95% CI: 0.27-0.39), P<0.001

Improved wound healing¹
91% patients with mCRC treated with FRUZAQLA + BSC experienced a Grade 2 event of wound dehiscence. Do not administer FRUZAQLA for at least 2 weeks prior to major surgery. Do not administer FRUZAQLA for at least 2 weeks after major surgery and until adequate wound healing. The safety of administration of FRUZAQLA after resumption of wound healing complications has not been established.

Arterial Thromboembolic Events. In 911 patients with mCRC treated with FRUZAQLA, 0.8% of patients experienced an arterial thromboembolic event. Initiation of FRUZAQLA in patients with a recent history of thromboembolic events should be carefully considered. In patients who develop arterial thromboembolism, discontinue FRUZAQLA.

Allergic Reactions to FDAC Yellow No. 6 (Tartrazine) and No. 8 (Sunset Yellow FCF). FRUZAQLA 1 mg capsules contain FDAC Yellow No. 6 (tartrazine), which may cause allergic-type reactions [including bronchial asthma] in certain susceptible persons. FRUZAQLA 1 mg capsules contain FDAC Yellow No. 8 (Sunset Yellow FCF), which may cause allergic reactions.

Embryo-Fetal Toxicity. Based on results from animal studies and its mechanism of action, FRUZAQLA can cause fetal harm when administered to pregnant women. Advise pregnant women of the potential risk to a fetus. Advise females of childbearing potential and males with female partners of childbearing potential to use effective contraception during treatment with FRUZAQLA and for 2 weeks after the last dose.

ADVERSE REACTIONS
The most common adverse reactions (incidence ≥20%) following treatment with FRUZAQLA included hypertension, palmar-plantar erythrodysesthesia (hand-foot skin reactions), proctocolitis, dyspnea, abdominal pain, diarrhea, and asthenia.

Footnotes use Rubik Regular in dark blue (7 pt)

References use Rubik Regular in dark blue (7 pt)

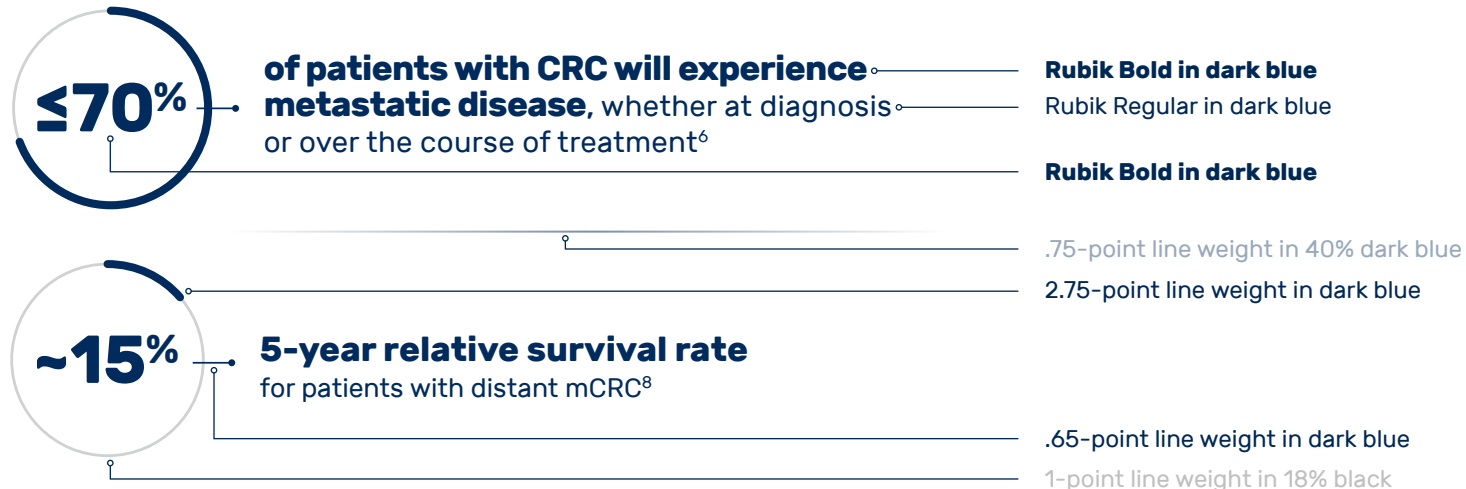
Sign-off 100% black (7-6 pt) depending on the piece. On this piece it is 6 pt

Logo is vertically centered with the sign-off.
Not base aligned

Space here between logo and sign-off copy should be 0.25". This might vary if the piece is very small

Unmet need

These are guidelines for unmet need baseline look and feel.



Sign-off

These are guidelines for applying the sign-off (end matter).



ONCOLOGY

TAKEDA® and the TAKEDA logo® are registered trademarks of Takeda Pharmaceutical Company Limited.
FRUZAQLA and  are trademarks of HUTCHMED Group Enterprises Limited, used under license.
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Rubik Regular in black

End matter should center align with Takeda ONCOLOGY logo



ONCOLOGY

TAKEDA® and the TAKEDA logo® are registered trademarks of Takeda Pharmaceutical Company Limited.
FRUZAQLA and  are trademarks of HUTCHMED Group Enterprises Limited, used under license.
HERE2ASSIST® and the HERE2ASSIST logo® are registered trademarks of Millennium Pharmaceuticals, Inc.
© 20XX Takeda Pharmaceuticals U.S.A., Inc. All rights reserved. US0-FRQ-XXXX XX/20XX

Rubik Regular in black

End matter should center align with Takeda ONCOLOGY logo

Color palette usage in digital assets

The Now Approved campaign uses the same colors across consumer and HCP audiences. However, the emphasis of certain colors may be dialed up or down to differentiate the look and feel in the absence of a finalized campaign, while maintaining brand cohesion.

HCP



For HCP materials, always use the purple as the primary, the light blue and green as the secondary, and the navy for text.



The HCP gradient in digital assets

begins with the purple at the beginning and progresses to blend into the blue, followed by the green.

Patient



For patient materials, always use the purple as the primary, the yellow as secondary, the light blue and green as tertiary, and navy for text.



The consumer gradient in digital assets

begins with the purple at the beginning and progresses to blend into the blue, followed by the green and yellow.

Microsites

The HCP and Patient Now Approved microsites introduce important information about FRUZAQLA. The differences between the HCP and Patient executions can be seen here.

HCP

The first and only novel targeted therapy approved for mCRC in more than a decade¹⁻⁴

NOW APPROVED
An innovation in mCRC

Express the FRUZAQLA Prescribing Information

Get FRUZAQLA sent to your office.
View Order Information

FRUZAQLA was proven effective for patients with previously treated mCRC¹

FRUZAQLA was studied in >1000 patients across 2 phase 3 clinical studies with consistent outcomes: FIRESCO and FIRESCO-2.¹

STUDY DESIGN FIRESCO-2 was an international, multicenter, double-blind, placebo-controlled, phase 3 study in cFV patients with previously-treated mCRC.¹

The primary endpoint was overall survival (OS).¹

7.4 months
(95% CI: 6.7-8.2)

4.8 months
(95% CI: 4.8-4.8)

34% reduction in risk of death
with FRUZAQLA + BSC vs placebo + BSC

— FRUZAQLA + BSC (n=461)
— Placebo + BSC (n=230)
HR: 0.56 (95% CI: 0.35-0.80);
P<0.001

The HCP and Patient sites emphasize different colors

Bolder graphics are preferred on the Patient site, while a more minimal style is preferred on HCP

Headline and button colors differ on each site

Charts are used to show information on the HCP site, while icons are preferred on the Patient site

Patient

FRUZAQLA™ (fruquintinib)

A new, once-daily innovative oral treatment for previously treated metastatic colorectal cancer (mCRC)

NOW APPROVED

FRUZAQLA is used to treat colorectal cancer (CRC) that has spread to other parts of the body. It is used by people who have received previous treatment with certain anti-cancer medicines.

About FRUZAQLA | Order Discussion Guide | Financial Assistance | Patient Resources

What is FRUZAQLA and how does it work?

FRUZAQLA (FIRESCO-2) works by starving tumors of nutrients and oxygen, which can stop tumor growth. It was designed to block proteins called vascular endothelial growth factor receptors (VEGFR). FRUZAQLA blocks blood supply from feeding the tumor.

FRUZAQLA is the first targeted therapy for mCRC approved in nearly a decade.

The efficacy and safety of FRUZAQLA versus placebo was studied across 2 clinical trials, which included more than 1000 people whose CRC had been previously treated.

Fruzaqla™
(fruquintinib) capsules
5 mg • 1 mg

HCP applications

The following examples show how the HCP look and feel is leveraged in the Now Approved banners and emails.

Banners



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Fruzaqla™
(fruquintinib) capsules
5 mg • 1 mg

with fluoropyrimidine-, oxaliplatin-, and irinotecan-based chemotherapy, an anti-VEGF therapy, and, if RAS wild-type, an anti-EGFR therapy.

INDICATIONS AND USAGE

Indication: FRUZAQLA is indicated for the treatment of patients with metastatic colorectal cancer who have been previously treated with fluoropyrimidine-, oxaliplatin-, and irinotecan-based chemotherapy, and, if RAS wild-type, an anti-EGFR therapy.

IMPORTANT SAFETY INFORMATION, WARNINGS AND PRECAUTIONS

- Hypertension** has been reported. Hypertension occurred in 49.4% of patients treated with FRUZAQLA. Grade 3-4 hypertension in 19.1% of patients (0.3%) experienced at least one episode of Grade 3-4 hypertension.

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Fruzaqla™
(fruquintinib) capsules
5 mg • 1 mg

[Explore the Data](#)

The "Now Approved" seal does not include the subheading

The preferred font for banner ads is Rubik

Primary CTAs use navy color with white text and should be rounded

Emails



Important Safety Information **Prescribing Information**

Fruzaqla™
(fruquintinib) capsules
5 mg • 1 mg

[Hi] [Dr.] [Preferred Name],

[Thank you for meeting with me, [NAME].]

Did you know the **FDA has recently approved (fruquintinib)** for patients with previously treated cancer (mCRC)? To schedule some time specific questions, please call me at [PHONE] or email me at [EMAIL ADDRESS].

Start learning about FDA-approved FRUZAQLA.

[Discover FRUZAQLA](#)

FDA APPROVAL IS HERE! ANNOUNCING FRUZAQLA

"For patients with mCRC previously treated with fluoropyrimidine-, oxaliplatin-, and irinotecan-based chemotherapy, an anti-VEGF therapy, and, if RAS wild-type, an anti-EGFR therapy."

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NOW APPROVED

Fruzaqla™
(fruquintinib) capsules
5 mg • 1 mg

FRUZAQLA™ is now approved.

For patients with metastatic colorectal cancer (mCRC) previously treated with fluoropyrimidine-, oxaliplatin-, and irinotecan-based chemotherapy, an anti-VEGF therapy, and, if RAS wild-type, an anti-EGFR therapy.

[Explore the Data](#)

FRUZAQLA was proven effective for patients with previously treated mCRC

FRUZAQLA was studied in a global, randomized, double-blind, placebo-controlled, multi-center, phase 3 study, FRESCO-2, in 691 patients with previously treated mCRC.

Median Overall Survival

7.4 months with FRUZAQLA + BSC (95% CI: 6.7-8.2)
vs 4.8 months with placebo + BSC (95% CI: 4.0-5.8)

[Review Efficacy](#)

Headlines are displayed in the navy color

Primary CTAs use the primary purple color with white CTA copy and should be squared off

The preferred font for copy on emails is the Arial family

Fruzaqla™
(fruquintinib) capsules
5 mg • 1 mg

ADA compliance/digital best practices

Below are common best practices to follow when creating any digital tactic in accordance with ADA compliance.

- Create a clear heading hierarchy with one H1
- Headings need to describe the topic of the content below them
- Color contrast for text that is 18 pt or 14 pt bold must have a ratio of at least 3:1; text that is regular 16 pt and below must have a ratio of 4.5:1
- Color contrast for graphic elements like button borders, radio buttons, etc, must have a ratio of at least 3:1
- Live text is always best!
- Never use color alone to convey information
- Use consistent styling across the site
- Ensure that interactive elements are easy to identify
- Include transcripts for both audio only and video content
- Add a pause/stop button for anything that automatically moves
- All interactive elements should be accessible by mouse and keyboard
- Site should have a 'skip to' link that allows the keyboard user to skip the navigation and move into the main content area
- All interactive elements need to have a focus indicator that is at least 2 CSS pixels thick around the perimeter of the element
- For video content, include closed captions
- The focus indicator must have a least a 3:1 contrast ratio between the same pixels in the focused and unfocused states (focus changing colors throughout the page is perfectly fine)
- Target size on touchscreens must be at least 24x24 CSS pixels
- Tables should be coded in HTML
- Images with meaning need a descriptive alt attribute
- Decorative images or icons need an empty alt attribute
- Design for different viewpoints
- CTA text should be descriptive and not duplicative of other CTAs on the same page
- Forms need to have clear instructions, with labels outside of the fields, and any changes of context (hidden extra steps) need to be communicated to the user
- Error messages need to be descriptive